

HEALTH SYSTEMS, PUBLIC HEALTH AND UNIVERSAL HEALTH COVERAGE

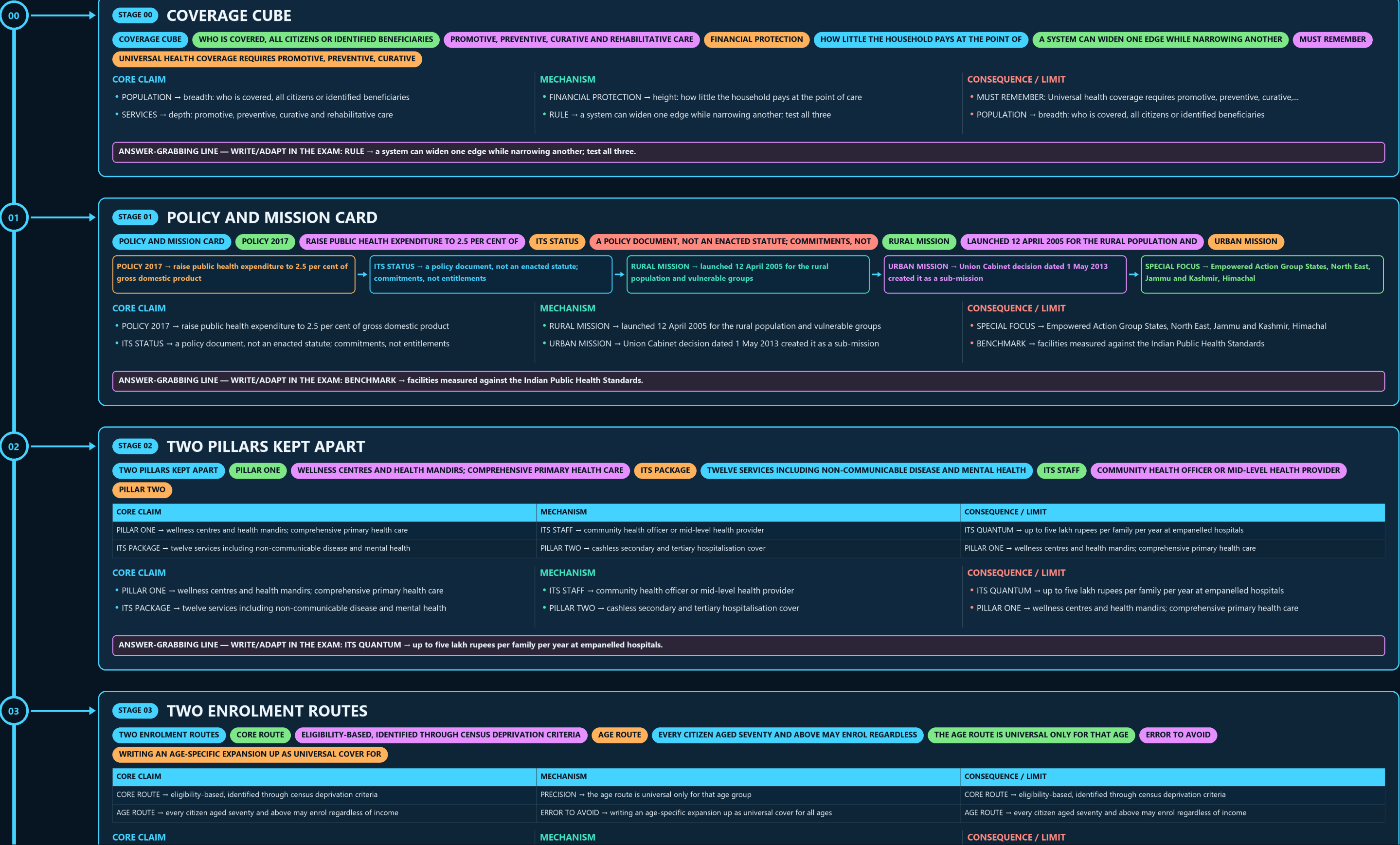
COVERAGE CUBE → POLICY AND MISSION CARD → TWO PILLARS KEPT APART → TWO ENROLMENT ROUTES → SOURCE DISCIPLINE CARD → IMPOVERISHMENT MECHANISM

Read the thick cyan rail from Stage 00 to the final core synthesis. Each card uses a concept-appropriate structure; the grey E card is optional enrichment only and never repairs missing core content.

PRIMARY CORE FLOW SUBORDINATE EXTRA APPROVAL / REVIEW

Approval: FALSE • Pending user review • source generation g3 and all prior artifacts unchanged

CORE BEFORE EXTRA • prior artefacts unchanged • exact same master drives poster and tiles



03

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals.

STAGE 03

TWO ENROLMENT ROUTES

TWO ENROLMENT ROUTES

CORE ROUTE

ELIGIBILITY-BASED, IDENTIFIED THROUGH CENSUS DEPRIVATION CRITERIA

AGE ROUTE

EVERY CITIZEN AGED SEVENTY AND ABOVE MAY ENROL REGARDLESS

THE AGE ROUTE IS UNIVERSAL ONLY FOR THAT AGE

ERROR TO AVOID

WRITING AN AGE-SPECIFIC EXPANSION UP AS UNIVERSAL COVER FOR

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
CORE ROUTE → eligibility-based, identified through census deprivation criteria	PRECISION → the age route is universal only for that age group	CORE ROUTE → eligibility-based, identified through census deprivation criteria
AGE ROUTE → every citizen aged seventy and above may enrol regardless of income	ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages	AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

CORE CLAIM

CORE ROUTE → eligibility-based, identified through census deprivation criteria

AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

MECHANISM

PRECISION → the age route is universal only for that age group

ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages

CONSEQUENCE / LIMIT

CORE ROUTE → eligibility-based, identified through census deprivation criteria

AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages.

04

STAGE 04

SOURCE DISCIPLINE CARD

SOURCE DISCIPLINE CARD

HEALTH AUTHORITY

IMPLEMENTS THE HOSPITALISATION PILLAR; EMPANELS AND PAYS CLAIMS

HEALTH ACCOUNTS

OFFICIAL ESTIMATES OF PUBLIC, PRIVATE AND OUT-OF-POCKET SPENDING

47.1 PER CENT IN 2019-20; 44.4 PER CENT IN

NEVER CONFUSE THE PUBLICATION DATE WITH THE ACCOUNTING YEAR

A HIGH SHARE SIGNALS WEAK PUBLIC PROVISION, NOT HIGH

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims	SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22	MEANING → a high share signals weak public provision, not high utilisation
HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending	RULE → never confuse the publication date with the accounting year	HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

CORE CLAIM

HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending

MECHANISM

SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22

RULE → never confuse the publication date with the accounting year

CONSEQUENCE / LIMIT

MEANING → a high share signals weak public provision, not high utilisation

HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → never confuse the publication date with the accounting year.

05

STAGE 05

IMPOVERISHMENT MECHANISM

IMPOVERISHMENT MECHANISM

CARE IS SOUGHT FROM A PROVIDER OUTSIDE THE FUNDED

OUT-OF-POCKET PAYMENT AT THE POINT OF SERVICE

CATASTROPHIC SPENDING

ABOVE A THRESHOLD SHARE OF HOUSEHOLD INCOME OR CONSUMPTION

ASSET SALE OR DEBT

YEARS OF ACCUMULATION ARE UNDONE BY ONE HOSPITALISATION

THIS IS WHERE THE HEALTH OWNER MEETS THE POVERTY

ILLNESS → care is sought from a provider outside the funded public layer

OUT-OF-POCKET PAYMENT at the point of service

CATASTROPHIC SPENDING: above a threshold share of household income or consumption

ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation

LINK → this is where the health owner meets the poverty owner

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
ILLNESS → care is sought from a provider outside the funded public layer OUT-OF-POCKET PAYMENT at the point of service	CATASTROPHIC SPENDING: above a threshold share of household income or consumption ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation	LINK → this is where the health owner meets the poverty owner CLOSE DISTINCTION: UHC is not free hospitalisation alone, insurance coverage is not...

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE DISTINCTION: UHC is not free hospitalisation alone, insurance coverage is not.

06

STAGE 06

REFERRAL SPINE

REFERRAL SPINE

FIRST CONTACT; OUTREACH AND COMMUNITY-LEVEL SERVICES

PRIMARY HEALTH CENTRE

PREVENTIVE, PROMOTIVE AND BASIC CURATIVE CARE

COMMUNITY HEALTH CENTRE

SPECIALIST OUTPATIENT AND INPATIENT CARE

DISTRICT HOSPITAL

SECONDARY CARE AND THE GATEWAY TO TERTIARY REFERRAL

PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care

SUB-CENTRE → first contact; outreach and community-level services

COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care

DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral

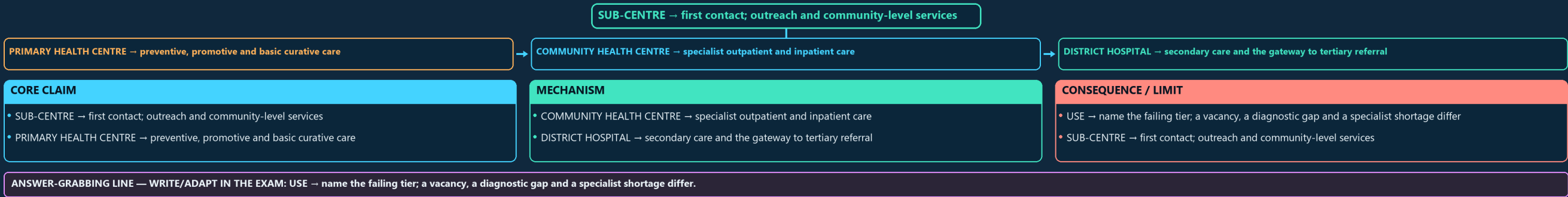
CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
SUB-CENTRE → first contact; outreach and community-level services	COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care	USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ

HEALTH SYSTEMS, PUBLIC HEALTH AND UNIVERSAL HEALTH COVERAGE • TILE 2/4 • same master rows 2572-5806 of 10950 • continues below

06

STAGE 06 REFERRAL SPINE

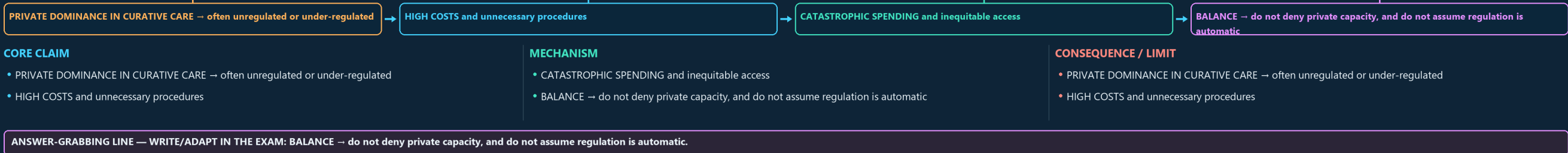
REFERRAL SPINE FIRST CONTACT; OUTREACH AND COMMUNITY-LEVEL SERVICES PRIMARY HEALTH CENTRE PREVENTIVE, PROMOTIVE AND BASIC CURATIVE CARE COMMUNITY HEALTH CENTRE SPECIALIST OUTPATIENT AND INPATIENT CARE DISTRICT HOSPITAL
SECONDARY CARE AND THE GATEWAY TO TERTIARY REFERRAL



07

STAGE 07 MARKETISATION AND ITS HARMS

MARKETISATION AND ITS HARMS PRIVATE DOMINANCE IN CURATIVE CARE OFTEN UNREGULATED OR UNDER-REGULATED HIGH COSTS AND UNNECESSARY PROCEDURES CATASTROPHIC SPENDING AND INEQUITABLE ACCESS DO NOT DENY PRIVATE CAPACITY, AND DO NOT ASSUME



08

STAGE 08 REGULATORY TOOLKIT

REGULATORY TOOLKIT CLINICAL ESTABLISHMENTS ACT, 2010 REGISTRATION AND REGULATION OF ESTABLISHMENTS ITS CONDITION STATES MUST ADOPT IT OR NOTIFY EQUIVALENT LEGISLATION; ADOPTION PRICE CAPPING EXERCISED OVER DEVICES BY THE PHARMACEUTICAL PRICING AUTHORITY
TREATMENT GUIDELINES

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments	PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority	RULE → name the adoption gap a regulatory recommendation is meant to close
ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments
CORE CLAIM - CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments - ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	MECHANISM - PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority - TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	CONSEQUENCE / LIMIT - RULE → name the adoption gap a regulatory recommendation is meant to close - CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments
ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → name the adoption gap a regulatory recommendation is meant to close.		

09

STAGE 09 FINANCING FAMILIES AND CHAIN

FINANCING FAMILIES AND CHAIN SOCIAL HEALTH INSURANCE CONTRIBUTION-LINKED POOLING, AS IN THE EMPLOYEES SYSTEM GENERAL REVENUE, AS IN THE HEALTH MISSION AND THE REVENUE COLLECTION TO RISK POOLING TO STRATEGIC PURCHASING TO
INSURANCE LEAVES PREVENTION, MEDICINES AND OUTPATIENT SPENDING UNRESOLVED CALLING THE TAX-FUNDED PILLAR A CONTRIBUTORY INSURANCE SCHEME

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	GAP → insurance leaves prevention, medicines and outpatient spending unresolved	SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system
CORE CLAIM - SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system - TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	MECHANISM - CHAIN → revenue collection to risk pooling to strategic purchasing to provision - GAP → insurance leaves prevention, medicines and outpatient spending unresolved	CONSEQUENCE / LIMIT - TRAP → calling the tax-funded pillar a contributory insurance scheme - SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system
ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.		

STAGE 09 FINANCING FAMILIES AND CHAIN

FINANCING FAMILIES AND CHAIN

SOCIAL HEALTH INSURANCE

CONTRIBUTION-LINKED POOLING, AS IN THE EMPLOYEES SYSTEM

GENERAL REVENUE, AS IN THE HEALTH MISSION AND THE

REVENUE COLLECTION TO RISK POOLING TO STRATEGIC PURCHASING TO

INSURANCE LEAVES PREVENTION, MEDICINES AND OUTPATIENT SPENDING UNRESOLVED

CALLING THE TAX-FUNDED PILLAR A CONTRIBUTORY INSURANCE SCHEME

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
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CORE CLAIM • SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system • TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	MECHANISM • CHAIN → revenue collection to risk pooling to strategic purchasing to provision • GAP → insurance leaves prevention, medicines and outpatient spending unresolved	CONSEQUENCE / LIMIT • TRAP → calling the tax-funded pillar a contributory insurance scheme • SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.

STAGE 10 COMMUNITY HEALTH MAPPING

COMMUNITY HEALTH MAPPING

REACH THE UNDERSERVED FIRST RATHER THAN THE EASIEST TO

VILLAGE HEALTH, SANITATION AND NUTRITION COMMITTEES

INTER-SECTORAL ACTION

WATER, SANITATION, NUTRITION AND GENDER EQUALITY

APPROPRIATE TECHNOLOGY

FRONTLINE HEALTH ACTIVISTS AND AUXILIARY NURSE MIDWIVES

THE UPGRADED WELLNESS CENTRE AS THE NAMED INDIAN DELIVERY

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
EQUITY → reach the underserved first rather than the easiest to reach	INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality	INSTITUTION → the upgraded wellness centre as the named Indian delivery point
PARTICIPATION → village health, sanitation and nutrition committees	APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	EQUITY → reach the underserved first rather than the easiest to reach
CORE CLAIM • EQUITY → reach the underserved first rather than the easiest to reach • PARTICIPATION → village health, sanitation and nutrition committees	MECHANISM • INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality • APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	CONSEQUENCE / LIMIT • INSTITUTION → the upgraded wellness centre as the named Indian delivery point • EQUITY → reach the underserved first rather than the easiest to reach

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: INSTITUTION → the upgraded wellness centre as the named Indian delivery point.

STAGE 11 CAPACITY LIMITS AND ANSWER SPINE

CAPACITY LIMITS AND ANSWER SPINE

UNION FINANCING AND STANDARDS DEPEND ON STATE DELIVERY CAPACITY

WORKFORCE VACANCIES AND UNEVEN CAPACITY LIMIT IDENTICAL NATIONAL SCHEMES

MENTAL HEALTH

A STATUTORY RIGHT AND A TELE-SERVICE STILL NEED DISTRICT

NAME THE COVERAGE DIMENSION TESTED; BUILD

PROVISION PLUS REGULATION

GRADED VERDICT WITH A DASHBOARD-COUNT CAVEAT AND A REVERSAL

CONDITION → Union financing and standards depend on State delivery capacity GAP → workforce vacancies and uneven capacity limit identical national schemes MENTAL HEALTH → a statutory right and a tele-service still need district professionals OPEN → name the coverage dimension tested; BUILD → provision plus regulation CLOSE → graded verdict with a dashboard-count caveat and a reversal condition				
CORE CLAIM • CONDITION → Union financing and standards depend on State delivery capacity • GAP → workforce vacancies and uneven capacity limit identical national schemes		MECHANISM • MENTAL HEALTH → a statutory right and a tele-service still need district professionals • OPEN → name the coverage dimension tested; BUILD → provision plus regulation	CONSEQUENCE / LIMIT • CLOSE → graded verdict with a dashboard-count caveat and a reversal condition • EVIDENCE LIMIT: AUTHORITY / IMPLEMENTATION: Map Union-state-local responsibilities,...	

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE → graded verdict with a dashboard-count caveat and a reversal condition.

EXTRA SUBORDINATE ENRICHMENT — ONLY AFTER THE COMPLETE CORE

OPTIONAL ADVANCED ENRICHMENT

EXPAND HWC/AAM FOR COMPREHENSIVE PRIMARY CARE — REDUCES NEED

STRENGTHEN NHM (FREE DRUGS, DIAGNOSTICS, WORKFORCE) — IMPROVES PUBLIC

PM-JAY FOR SECONDARY/TERTIARY COVER — SHIFTS HOSPITALISATION COST FROM

REGULATORY ENFORCEMENT

CLINICAL ESTABLISHMENTS ACT, PRICE CAPS (NPPA FOR

PUBLIC-PRIVATE PARTNERSHIPS WITH ACCOUNTABILITY

EMPANEL PRIVATE PROVIDERS

OPTIONAL SOURCE DEPTH • Expand HWC/AAM for comprehensive primary care — reduces need for • Strengthen NHM (free drugs, diagnostics, workforce) — improves public • PM-JAY for secondary/tertiary cover — shifts hospitalisation cost from • Regulatory enforcement: Clinical Establishments Act, price caps (NPPA for	ADVANCED DEBATE • Public-private partnerships with accountability: empanel private providers • NHP 2017: Policy, not statute; sets vision for UHC, primary-care-led • NHM (NRHM + NUHM): Centrally-sponsored scheme; provides flexi-pool funds, • Ayushman Bharat - HWC/AAM: Target: 1.5 lakh HWCs (upgraded SCs/PHCs);	LEGACY / NUANCE • Ayushman Bharat - PM-JAY: Its core route uses notified eligibility criteria • CAUTION: Clinical Establishments Act adoption: Some states have adopted or enacted • 2024 Q17: The question directly tests the state's role against • PM-JAY 70+ expansion: The all-citizen 70+ route is operational, regardless of
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OPTIONAL STATUS — This optional depth is unnecessary for a competent core answer; use it only for added nuance after the complete core has been written.